

* XR Report - Auth (Verified) - Internal Provenance Source: Pacific Neurosurgery & Spine Medical Group Inc; Garcia, Mia *



Patient Name: MARTINEZ, LELIA
Date of Birth: 11/09/1948
Account/ID: MALE2CV0
Study Date: 03/18/2024
Referral By: Sanjay Ghosh, M.D.

SMI Imaging Center of San Diego, LLC
Dr. Sanjay Ghosh
7625 Mesa College Dr. Suite 110A
San Diego, CA 92111
Phone: (619)810-1010
Fax: (619)810-1011

PROCEDURE: MR Cervical (MRI SPINE CERV W/O CO)

HISTORY:

Neck pain radiating into the right shoulder with concern for radiculopathy.

TECHNIQUE:

Multisequence, multiplanar imaging of the cervical spine performed without the use of intravenous contrast including sagittal T2, sagittal STIR, sagittal T1 as well as axial T2 and gradient echo imaging.

COMPARISON:

Same day cervical spine radiographs, separately reported

FINDINGS:

Background of mild congenital spinal canal narrowing. The cervical lordosis maintained. Intraosseous hemangioma dorsally at C5 and within the left lateral aspect of T4. Mild Modic type 1 endplate changes at T1-T2. Craniocervical alignment is maintained. The visible contents of the posterior fossa are normal. The cervical vertebral and visible carotid arterial flow voids are maintained. No superficial or deep neck space mass identified. Additional degenerative findings level by level are indexed below:

C2-C3: 1 mm anterolisthesis with mild disc desiccation. 2 mm central disc protrusion. No significant thecal sac narrowing. Moderate left and mild right facet hypertrophic change. Neural foramina remain patent.

C3-C4: Mild disc desiccation. Small posterior disc bulge with dorsal annular fissuring. Minimal thecal sac narrowing. Moderate left and mild right facet hypertrophic change. Left asymmetric uncovertebral hypertrophy. Mild left and no significant right neural foraminal narrowing.

C4-C5: Mild disc desiccation and height loss. Retrolisthesis. Broad-based 2 mm central disc protrusion. In combination with infolding there is moderate acquired canal stenosis. Moderate left-greater-than-right facet hypertrophic change. Left asymmetric uncovertebral hypertrophy. Mild left-greater-than-right neural foramina narrowing.

C5-C6: Mild disc desiccation. Broad-based posterior disc osteophyte complex with mild acquired canal stenosis. Mild facet hypertrophic change. Left asymmetric uncovertebral hypertrophy. Mild left and no significant right neural foramina narrowing.

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Continued:

C6-C7: Mild disc desiccation. 2 mm left central disc protrusion. In combination ligamentum flavum infolding there is mild acquired canal stenosis. Left asymmetric uncovertebral hypertrophy. Moderate left and no significant right neural foramina narrowing.

C7-T1: Mild disc desiccation. No acquired canal stenosis. Mild-to-moderate facet hypertrophic change. Neural foramina are patent.

T1-T5: Sagittal sequences only. No high-grade canal or neural foraminal narrowing.

IMPRESSION:

1. Background of mild congenital spinal canal narrowing with superimposed degenerative disc disease most pronounced at C4-C5 where retrolisthesis is associated with a 2 mm central disc protrusion and moderate acquired canal stenosis. No high-grade spinal canal narrowing or frank cord compression any level.

2. Multilevel uncovertebral and facet hypertrophic change resulting in various degrees of neural foramina narrowing, up to moderate in severity on the left at C6-C7. No high-grade foraminal stenosis or frank impingement upon the exiting cervical nerve roots.

3. Mild Modic type 1 endplate changes at T1-T2.

Interpreted By: Marin McDonald, M.D.

Electronically Signed By: Marin McDonald, M.D.

Electronically Signed On: 2024-03-19 07:04 -07:00 [ID:1986873][IL:92103]

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PROCEDURE: CR Cervical (SPINE CERV 4V)

HISTORY:
Neck pain.

TECHNIQUE:
Four views cervical spine including lateral weight-bearing flexion, extension, neutral position, and AP weight-bearing

FINDINGS:
Cranial-cervical junction, C1-C2, C2-C3, C3-C4, normal

C4-C5, mild disc space narrowing with small marginal osteophytes

C5-C6, small marginal osteophytes without significant disc space narrowing

C6-C7, C7-T1 are within normal limits.

No significant facet arthropathy. No abnormalities of alignment with flexion, extension, neutral position.

Prevertebral soft tissues normal.

IMPRESSION:
Multilevel cervical spondylosis as described above.

Interpreted By: Derek Burdeny, M.D.
Electronically Signed By: Derek Burdeny, M.D.
Electronically Signed On: 2024-03-19 06:20 -07:00 [ID:1987662][IL:68144-5611]